



REF. DOCTOR : demo@tbvision.com

AGE/SEX : 45 Years

DRAWN : 29/03/2026 12:23:29

PATIENT : John Doe | ID: #923866

RECEIVED : 29/03/2026 12:23:29

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Test Report Status	Final	Results	Reference Interval	Units
AI-BASED TUBERCULOSIS DIAGNOSTIC ANALYSIS (TBFERON-AI)				
SPECIMEN SOURCE		MULTIMODAL INPUT		
METHOD : DIGITAL CHEST X-RAY + CLINICAL DATA				
CNN X-RAY PROBABILITY		10.5%	< 40.0%	%
METHOD : DEEP LEARNING - MOBILENETV2				
CLINICAL PARAMETER SCORE		25.0%	< 40.0%	%
METHOD : LOGISTIC REGRESSION ML MODEL				
FINAL FUSED TB PROBABILITY		16.3% Low	< 40.0%	%
METHOD : WEIGHTED MULTIMODAL FUSION (60% IMAGE + 40% CLINICAL)				
COUGH DURATION		2 Weeks	> 2 Weeks indicates risk	Weeks
METHOD : CLINICAL PARAMETER				
FEVER / WEIGHT LOSS / NIGHT SWEATS		Yes / No / No		
METHOD : CLINICAL PARAMETER				
SPUTUM SMEAR MICROSCOPY		Not Done	NEGATIVE	
METHOD : LAB DATA / ACID-FAST BACILLI				
GENEXPERT MTB/RIF		Not Done	NEGATIVE	
METHOD : LAB DATA / NUCLEIC ACID AMPLIFICATION				
INTERPRETATION		NEGATIVE	NEGATIVE	
METHOD : AI MULTIMODAL FUSION ANALYSIS				

Interpretation(s)

The AI-based TB detection system uses a multimodal approach combining Deep Learning analysis of the Chest X-Ray image with clinical parameter scoring (cough duration, fever, weight loss, night sweats) to arrive at a fused probability score.

AI Recommendation: TB unlikely. Consider other respiratory conditions if symptoms persist.

DISCLAIMER: This report is generated by an AI system for clinical decision support only. It is not intended to replace professional medical diagnosis. A qualified physician should evaluate these results in the context of the patient's full clinical picture. Confirmatory tests (sputum culture, GeneXpert, TST) are recommended for definitive TB diagnosis.

Authorized Signatory

TB-Vision AI Diagnostic System

Reviewing Physician

demo@tbvision.com



